EMS CLINICAL CHECKLISTS — BATCH 3

Paramedic level with EMT delegation · 2001 Resuscitation Guidelines / Field Termination · 4003 Asthma / Emphysema / COPD · 4012 Overdose / Poisoning · 4013 Respiratory Distress / Pulmonary Edema · 4014 Seizures

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

EDITION CHECK REQUIRED BEFORE ANY USE. The source file's cover says 'Last Update 07/01/2026'. Appendix A (PDF p.321) shows the 2026-2028 changes as Effective Nov. 1, 2026. 1001.1.h: after release of a new version, all previous versions are invalid. Confirm which edition is in force with agency clinical leadership (G-1).

SOURCE

Greater Miami Valley EMS Council standing orders, Regional Physician Advisory Board Region 3, file 2026protocol.pdf (328 pages). Page numbers = PDF viewer page index (1-328). Each protocol also prints its own 'Page x of y' footer; the printed footer is NOT used. Section numbers (e.g., 2008.2) are cited with every item.

CONTENTS OF THIS BATCH

Protocol	Title	Effective	Last modified	PDF pages used	Checkboxes	Not-specified lines
2001	Resuscitation Guidelines and Field Termination	June 1, 2021	May 26, 2026	PDF p.45 (+ 1003 p.17, 1005 p.21; Appendix A p.321)	20	4
4003	Asthma / Emphysema / COPD	June 1, 2021	May 16, 2026	PDF pp.121-122 (+ monographs 8002 p.227, 8017 p.257, 8018 p.259, 8023 p.269, 8026 p.275, 8029 p.281, 8030 p.283; Appendix A pp.322-323)	27	4
4012	Overdose / Poisoning	June 1, 2021	Mar. 9, 2025	PDF pp.141-142 (+ monographs 8006 p.235, 8031 p.285, 8033 p.289, 8034 p.291, 8042 p.307)	32	6
4013	Respiratory Distress / Pulmonary Edema	June 1, 2021	May 16, 2026	PDF p.143 (+ monograph 8034 p.291, 8002 p.227, 8023 p.269)	18	3
4014	Seizures	June 1, 2021	May 16, 2026	PDF pp.145-146 (+ 1005 p.22; monographs 8024 pp.271-272, 8029 p.281, 8031 p.285; Appendix A pp.322-323)	27	4

HOW TO READ THE TAGS

Tag	Meaning (source)
MCP ORDER	Source marks the item with a diamond. Per 1001.5.d.iv, diamond items ALWAYS require a physician's order (PDF p.12).
{ } OPTION	Source brackets the item. Per 1001.5.d.v, bracketed items are at the option of the agency and its Medical Director (PDF p.12).
EMR+ / EMT+ / AEMT+ / PM	Lowest level at which the source's colored level table lists the item. Levels are cumulative: the table lists directives 'for the EMT and above', etc. (1001.6, PDF p.13).
A / P / G	Source markers: A = adult only, P = pediatric (pink), G = geriatric (1001.5.d.vi-viii, PDF p.12).

WHAT THE DIAMOND AND BRACE MARKS DO NOT TELL YOU

Your department's Medical Director decides which { } options are adopted (1001.4.d: nothing in the protocol may be used without specific pre-approval of the local Medical Director). 1001.4.e-f: the protocol is used as clinically indicated; not every standing order applies to every patient, and discretionary judgment is required.

STRUCTURE OF THIS FILE

Cover; five checklists in the order above (each starts with its own title bar and status banner and follows the seven-heading template); REQUIRES HUMAN VERIFICATION (full text of every gap and conflict); SOURCE-VERIFICATION TABLES (one row per checklist line). Use the companion HTML file for collapsible, tappable checklists.

AUDIT SUMMARY

Automated source audit at build: 137 checklist items in this file - every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures; whole-document audit 472 items, 0 failures); 157 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 21 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

Contrast check of the print palette (WCAG ratio): navy on white 11.48:1; body on white 17.06:1; teal on white 4.99:1; red on white 5.44:1; cream on navy 10.42:1; white on navy 11.48:1.

CARDIAC PROTOCOL 2001 — RESUSCITATION GUIDELINES AND FIELD TERMINATION

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Resuscitation Guidelines and Field Termination	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2001 — Resuscitation Guidelines and Field Termination	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 26, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.45 (+ 1003 p.17, 1005 p.21; Appendix A p.321)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

GENERAL GUIDELINES (2001.1)

☐	**ALL**	A patient's BEST CHANCE for resuscitation is at the scene with high quality CPR and code management.	2001.1.a · p.45
☐	**PM**	Paramedics are expected to provide ALS resuscitative care at the scene.	2001.1.b · p.45
☐	**ALL**	All providers should prioritize on-scene resuscitation efforts, regardless of age, unless clinically indicated. *Condition / note: 'Unless clinically indicated' is not defined (see U-2001-7).*	2001.1.c · p.45
☐	**ALL**	These standards apply to all patient populations, including both adults and pediatrics.	2001.1.d · p.45

PRESENTATION (2001.2)

☐	**EMR+**	Signs: pulseless and apneic; does NOT meet the Non-initiation of Care Guideline. *Condition / note: Non-initiation criteria are in 1003 (Batch 1).*	2001.2 Signs & Symptoms · p.45
	ALL	Listed differential: meets Non-initiation of Care Guideline.	2001.2 Differential Diagnosis · p.45
☐	**ALL**	PEDIATRIC: FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS. *Condition / note: Age boundary vs the 18-year criterion: see U-2001-1.*	2001.2 Pediatric Considerations · p.45

2 ASSESSMENT

Rhythm check, CPR quality targets, airway / IV / IO steps and arrest assessment details are NOT in 2001 — NOT SPECIFIED IN SOURCE — VERIFY (see 2002, 2003 and 2005).

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL

☐	**EMR+**	The EMR will continue resuscitation until the patient is handed off to a higher-level provider.	2001.2 Tx Algorithm, EMR block · p.45

EMT LEVEL AND ABOVE

☐	**EMT+**	The EMT will continue resuscitation until the patient is handed off to a higher-level provider.	2001.2 Tx Algorithm, EMT block · p.45

AEMT LEVEL AND ABOVE

☐	**AEMT+**	Patients will require PROLONGED resuscitation efforts if: they have PEA with a rate greater than 40 per minute; OR they have an upward trending or persistent EtCO2 greater than or equal to 20 mmHg, refractory to VF or VT. *Condition / note: See U-2001-3.*	2001.2 Tx Algorithm, AEMT block · p.45
	ALL	Pearl: there are situations where resuscitation may take 30 minutes or more.	2001.2 Clinical Pearls · p.45
	ALL	Pearl: in pseudo PEA, the patient may not be in true cardiac arrest, but simply not have palpable pulses due to profound shock.	2001.2 Clinical Pearls · p.45

EMT DELEGATION — from the source level tables

MAY	EMR and EMT: continue resuscitation until the patient is handed off to a higher-level provider (2001.2 EMR / EMT blocks, PDF p.45).
MAY	EMT: if no higher-level provider is available, transport; transport a patient with ROSC to an interventional facility if transport time is less than 30 minutes (EMT block).
MCP ORDER	EMT, MCP ORDER (diamond): if no ALS equipment is available at the scene and transport time to a medical facility will exceed 20 minutes, field termination may be considered (EMT block). The full field-termination criteria and the MCP-contact script (AEMT or Paramedic) are AEMT-block / Consult items.

MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Prolonged-resuscitation criteria; profound-hypothermia transport rule; the six-point field-termination criteria; contacting MCP for field-termination consent (AEMT block / Consult 'A' marker).

4 REASSESSMENT / RESPONSE

FIELD TERMINATION: DECISION CRITERIA

☐	**AEMT+ MCP ORDER**	MCP ORDER: following all appropriate efforts, field termination requires MCP approval and may ONLY be considered when ALL of these are met: 18 years or older; in asystole or PEA with rates less than 40; NOT in arrest due to hypothermia; advanced airway in place; vascular access in place; no signs of neurological function such as reactive pupils, response to pain or spontaneous movement. *Condition / note: 'All appropriate efforts' is not defined; PEA exactly 40 is not covered (see U-2001-3, U-2001-4).*	2001.2 Tx Algorithm, AEMT block · p.45
☐	**EMT+ MCP ORDER**	MCP ORDER (EMT block): if NO ALS equipment is available at the scene and transport time to a medical facility will exceed 20 minutes, field termination MAY be considered. *Condition / note: Relationship to the AEMT-block criteria is not stated (see U-2001-2).*	2001.2 Tx Algorithm, EMT block · p.45

Reassessment interval, number of resuscitation cycles or minimum duration before stopping, and post-ROSC reassessment: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

TRANSPORT

☐	**EMT+**	If no higher-level provider is available, then transport.	2001.2 Tx Algorithm, EMT block · p.45
☐	**EMT+**	Patient with ROSC should be transported to an interventional facility if transport time is less than 30 minutes.	2001.2 Tx Algorithm, EMT block · p.45
☐	**AEMT+**	If arrest is due to profound hypothermia, rapidly transport to a Trauma Center. *Condition / note: 'Profound' is not defined (see U-2001-6).*	2001.2 Tx Algorithm, AEMT block · p.45
☐	**PM**	Transport to a cardiac interventional facility if transport time is less than 30 minutes: a documented STEMI and a witnessed cardiac arrest; a return of spontaneous circulation (ROSC).	2001.2 Tx Algorithm, Paramedic block · p.45
	ALL	Pearl: consider aeromedical transport for transports greater than 30 minutes if the patient has ROSC. Research has shown that CPR quality diminishes while being transported.	2001.2 Clinical Pearls · p.45

CONSULT: INFORMATION TO GIVE MCP FOR FIELD TERMINATION CONSENT (AEMT OR PARAMEDIC, DIRECT CONTACT)

☐	**AEMT+**	The duration of the resuscitation.	2001.2 Consult · p.45
☐	**AEMT+**	How long the patient may have been in arrest prior to EMS arrival.	2001.2 Consult · p.45
☐	**AEMT+**	Whether it was a witnessed or unwitnessed event.	2001.2 Consult · p.45
☐	**AEMT+**	The current EtCO2. *Condition / note: EtCO2 is reported but is not a stated termination criterion.*	2001.2 Consult · p.45
☐	**AEMT+**	The presenting rhythm.	2001.2 Consult · p.45

Receiving-facility notification and handoff content for arrest: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for resuscitation and field termination (e.g., time of death, MCP name and time, criteria met): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2001-1	2001.2 Pediatric vs criteria (p.45)	'FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS', yet the criteria say '18 years or older'
U-2001-2	2001.2 EMT vs AEMT blocks (p.45)	Two field-termination provisions: (1) EMT block, diamond: 'If no ALS equipment is available at the scene, and...
U-2001-3	2001.2 AEMT block (p.45)	'Prolonged resuscitation' if PEA 'greater than 40 per minute'; field termination only for asystole or PEA 'less than...
U-2001-4	2001.2 AEMT block (p.45)	'Following all appropriate efforts' has no minimum duration, cycle count or intervention list in 2001; those are in...
U-2001-5	2001.2 Tx Algorithm (p.45)	ROSC and STEMI-with-witnessed-arrest go to an interventional facility if transport time is less than 30 minutes;...
U-2001-6	2001.2 AEMT block (p.45)	'Profound hypothermia' is undefined and the destination is a Trauma Center; 1003.3.a excludes hypothermic patients...
U-2001-7	2001.1.c / 2001.2 (p.45)	'Prioritize on-scene resuscitation ... unless clinically indicated' — no list of clinical indications to transport...
U-2001-8	2001 edition (p.45)	'Last modified' is May 26, 2026 (the other Batch 2 protocols show May 16, 2026)

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4003 — ASTHMA / EMPHYSEMA / COPD

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Asthma / Emphysema / COPD	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4003 — Asthma / Emphysema / COPD	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.121-122 (+ monographs 8002 p.227, 8017 p.257, 8018 p.259, 8023 p.269, 8026 p.275, 8029 p.281, 8030 p.283; Appendix A pp.322-323)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (4003.1)

☐	**EMR+**	Signs & symptoms: shortness of breath; pursed lip breathing; increased respiratory rate and effort; wheezing, rhonchi; accessory muscle use; cough; tachycardia; tripod position.	4003.1 Signs & Symptoms · p.121
	ALL	Listed differential: anaphylaxis; aspiration; pleural effusion; pneumonia; pulmonary embolus; pneumothorax; cardiac event (AMI or CHF); pericardial tamponade; hyperventilation; inhaled toxins.	4003.1 Differential Diagnosis · p.121
☐	**ALL**	PEDIATRIC: younger patients may exhibit nasal flaring. Epinephrine is dosed based on weight, not age.	4003.1 Pediatric Considerations · p.121
☐	**ALL**	PEDIATRIC: the protocol lists patients under 15 kg as pediatric, but patients equal to or greater than 30 kg will get Epinephrine 0.5 mg IM, no matter what their age. *Condition / note: See U-4003-3.*	4003.1 Pediatric Considerations · p.121

2 ASSESSMENT

Required assessment elements (history, medication use, vital signs, SpO2 / EtCO2 targets) and severity criteria, including what makes 'severe distress': NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	Provide O2 as needed.	4003.1 Tx Algorithm, EMR block · p.121

EMT LEVEL AND ABOVE

☐	**EMT+**	If the patient develops wheezing, assist with taking their prescribed metered dose inhaler OR consider Albuterol 2.5 mg and Ipratropium 0.5 mg, nebulized with O2 flowing at 8-10 LPM. *Condition / note: No MCP order is printed (Appendix A: requirement removed). 4002 still prints one (see U-4003-1).*	4003.1 Tx Algorithm, EMT block · p.121 8002 Adult Dosing · p.227 8023 Adult Dosing · p.269
☐	**EMT+**	For any patient who is bronchial constricted: CPAP. If indicated, consider nebulized Albuterol 2.5 mg and Ipratropium 0.5 mg in line with CPAP. *Condition / note: CPAP settings / contraindications are not in 4003 (see U-4003-2).*	4003.1 Tx Algorithm, EMT block · p.121

AEMT LEVEL AND ABOVE

☐	**AEMT+**	If the patient is intubated, Albuterol 2.5 mg by nebulizer into the ETT. If Ipratropium was not given before intubation, add it to the first Albuterol.	4003.1 Tx Algorithm, AEMT block · p.121
☐	**AEMT+**	After intubation of an asthma patient, limit the ventilation rate to avoid auto-PEEP and hypotension, provided you can adequately oxygenate the patient: ADULT 8-10 breaths per minute; PEDIATRIC 10-15 breaths per minute.	4003.1 Tx Algorithm, AEMT block · p.121
☐	**AEMT+**	Consider needle decompression in the presence of auto-PEEP or hyperinflation. If the patient is in cardiac arrest, perform BILATERAL needle decompression. If unilateral or bilateral diminished breath sounds and the patient is hemodynamically unstable, consider decompression of only the affected sides. *Condition / note: Level read from the green color bar (see U-4003-4).*	4003.1 Tx Algorithm, AEMT block · p.121
☐	**AEMT+**	Decompression sites: fourth or fifth intercostal space in the mid-axillary line; OR second or third intercostal space in the mid-clavicular line (use nipple line as a guide). PEDIATRIC less than 8 years old: limited to the 2nd or 3rd intercostal space at the mid-clavicular line.	4003.1 Tx Algorithm, AEMT block · p.121
☐	**AEMT+**	Asthmatics in severe distress (NOT for emphysema or COPD): if equal to or greater than 30 kg, give both Adult EpiPen and EpiPen Jr OR Epi (1:1,000) 0.5 mg IM (adult and pediatric). *Condition / note: EMT may not treat asthma with epinephrine (8017 / 8018). 'Severe distress' undefined (see U-4003-3).*	4003.1 Tx Algorithm, AEMT block · p.121 8018 Indications / Adult Dosing · p.259 8017 Indications · p.257
☐	**AEMT+**	PEDIATRIC: equal to or greater than 15 kg and less than 30 kg, Adult EpiPen OR Epinephrine (1:1,000) 0.3 mg IM. Less than 15 kg, EpiPen Jr OR Epinephrine (1:1,000) 0.15 mg IM.	4003.1 Tx Algorithm, AEMT block · p.121 8018 Pediatric Dosing · p.259

PARAMEDIC

☐	**PM**	If a conscious patient requires intubation — ADULT: Lidocaine 100 mg IN, half dose per nostril, or added to nebulizer with breathing treatment. PEDIATRIC: Lidocaine 1.5 mg/kg nebulized with O2 8-10 LPM or IN; maximum dose 100 mg.	4003.1 Tx Algorithm, Paramedic block · p.121 8026 Adult / Pediatric Dosing · p.275
☐	**PM** **{ } OPTION**	For any persistent bronchial constriction consider CPAP or {Bi-PAP}.	4003.1 Tx Algorithm, Paramedic block · p.121
☐	**PM**	Solu-Medrol: ADULT 125 mg IV. PEDIATRIC 2 mg/kg IV, max dose 125 mg.	4003.1 Tx Algorithm, Paramedic block · p.121 8030 Adult / Pediatric Dosing · p.283
☐	**PM** **MCP ORDER**	MCP ORDER: bronchoconstriction due to asthma refractory to Albuterol, Ipratropium, and Epinephrine — ADULT Magnesium Sulfate 2 gm (half bag) infused with macro-drip tubing over 10 minutes. *Condition / note: 8029 states refractory to 'Duo-Neb, Epinephrine and Solu-Medrol' (see U-4003-6).*	4003.1 Tx Algorithm, Paramedic block · p.121 4003.1 Consult · p.122 8029 Adult Dosing / Medical Control · p.281
	ALL	Pearl: no significant change in patient condition in the field should be expected from the administration of Solu-Medrol.	4003.1 Clinical Pearls · p.122

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 4003 ITSELF)

☐	**EMT+**	ALBUTEROL (8002): prior hypersensitive reaction to albuterol; cardiac dysrhythmias associated with tachycardia. IPRATROPIUM (8023): contraindications none in the emergency setting; caution in narrow-angle glaucoma and lactating mothers. Once initiated, the patient should be removed by EMS (8002, 8023).	8002 Contraindications / Precautions · p.227 8023 Contraindications / Precautions · p.269
☐	**AEMT+**	EPINEPHRINE (8017, 8018): contraindications none in the emergency setting. Side effects include dysrhythmias (VT and VF), hypertension, tachycardia, increased myocardial oxygen demand or precipitation of angina pectoris; syncope has occurred following epinephrine administration to asthmatic children.	8018 Contraindications / Precautions · p.259 8017 Contraindications · p.257
☐	**PM**	MAGNESIUM SULFATE (8029): hypersensitivity; any heart block except first degree; administer with caution if SBP less than 90 mmHg; continuous cardiac monitoring and frequent vital signs required; can cause respiratory depression.	8029 Contraindications / Precautions · p.281
☐	**PM**	SOLU-MEDROL (8030): contraindications none in emergency setting; do not initiate an IV only to administer this medication.	8030 Contraindications / Precautions · p.283
☐	**PM**	LIDOCAINE (8026): hypersensitivity; second or third degree heart block in absence of an artificial pacemaker; extreme caution with hepatic disease, heart failure, marked hypoxia, severe respiratory depression, hypovolemia or shock.	8026 Contraindications / Precautions · p.275

EMT DELEGATION — from the source level tables

MAY	EMT: provide O2 as needed; call for transport; assist the patient with their prescribed metered dose inhaler; consider Albuterol 2.5 mg + Ipratropium 0.5 mg nebulized with O2 at 8-10 LPM, may repeat Albuterol two times; CPAP for any bronchial-constricted patient, with nebulized treatment in line if indicated; transport unless ALS intercept is less than 5 minutes (4003.1 EMR / EMT blocks, PDF p.121). No MCP order is printed for the EMT in 4003 (Appendix A p.322: requirement removed; 8002 / 8023 Medical Control: No).
NOTE	EMT and epinephrine: the EMT may NOT treat asthma with epinephrine (8017 and 8018 Indications). Asthma epinephrine is AEMT-block. For the same nebulized medications, 4002 (allergic reaction) still prints an EMT MCP-order line — see U-4002-1 and U-4003-1.

MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Epinephrine for asthma; nebulized albuterol into the ETT; post-intubation ventilation rate; needle decompression; lidocaine; Solu-Medrol; magnesium sulfate; Bi-PAP.

4 REASSESSMENT / RESPONSE

☐	**EMT+**	EMT: may repeat Albuterol 2.5 mg nebulized two times.	4003.1 Tx Algorithm, EMT block · p.121 8002 Adult Dosing · p.227
☐	**AEMT+**	Asthmatics in severe distress: may repeat Epinephrine (1:1,000) 0.5 mg IM after 10 minutes (adult and pediatric); pediatric at weight appropriate dose after 10 minutes. *Condition / note: Maximum number of doses not stated (see U-4003-3).*	4003.1 Tx Algorithm, AEMT block · p.121

Reassessment interval, response-to-treatment criteria and criteria that define 'refractory' bronchoconstriction (how many treatments, how long): NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport.	4003.1 Tx Algorithm, EMR block · p.121
☐	**EMT+**	Transport unless ALS intercept is less than 5 minutes.	4003.1 Tx Algorithm, EMT block · p.121
☐	**ALL**	A patient who has received a breathing treatment should be transported for evaluation.	4003.1 Clinical Pearls · p.122
☐	**PM MCP ORDER**	Paramedic needs MCP orders to administer Magnesium Sulfate to the asthmatic patient.	4003.1 Consult · p.122

Destination, notification and handoff content for respiratory distress: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for asthma / COPD (e.g., treatments given before arrival, refusal after breathing treatment): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4003-1	4003.1 vs 4002.2 (p.122)	4003 prints no MCP-order requirement for the EMT's nebulized Albuterol / Ipratropium, consistent with Appendix A...
U-4003-2	4003.1 EMT / Paramedic (p.121)	CPAP is listed for 'any patient who is bronchial constricted' (EMT) and Bi-PAP is a bracketed { } option (Paramedic)
U-4003-3	4003.1 AEMT block vs 8017 / 8018 (p.259)	Epinephrine: 'Asthmatics in severe distress (NOT for emphysema or COPD)' — 'severe distress' is not defined; the...
U-4003-4	4003.1 AEMT block (p.121)	Needle decompression: 'consider ... in the presence of auto-PEEP or hyperinflation' — how these are recognized is...
U-4003-5	4003.1 AEMT block (p.121)	Post-intubation ventilation rate (adult 8-10 / pediatric 10-15 per minute) is conditional on being able to...
U-4003-6	4003.1 vs 8029 (p.281)	Magnesium: 4003 says 'refractory to Albuterol, Ipratropium, and Epinephrine' (adult marker only, diamond, MCP order)
U-4003-7	4003.1 Paramedic block (p.121)	Solu-Medrol is listed without timing relative to other treatments or an indication restriction (8030: asthma, COPD,...
U-4003-8	4003 Appendix A (p.322)	Appendix A (pp.322-323) lists new 4003 text for the EMT (no MCP for nebulized treatments; in-line with CPAP)

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4012 — OVERDOSE / POISONING

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Overdose / Poisoning	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4012 — Overdose / Poisoning	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Mar. 9, 2025
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.141-142 (+ monographs 8006 p.235, 8031 p.285, 8033 p.289, 8034 p.291, 8042 p.307)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

GUIDELINES (4012.1)

☐	**ALL** **MCP ORDER**	MCP ORDER (diamond): EMS personnel should contact and confer with MCP for direction on suspected poisonings. *Condition / note: Timing relative to naloxone is not stated (see U-4012-1).*	4012.1.a · p.141 4012.2 Consult · p.142
☐	**ALL**	Poison Control is intended for use by the general public.	4012.1.b · p.141
☐	**ALL**	If possible, provide the receiving facility all available information about the substance: safety data sheets (SDS); the container (if it is safe to do so); the label or an image of the label and warning information if it is unsafe or impractical to transport the container.	4012.1.c · p.141
☐	**EMR+**	Signs & symptoms: mental status changes; hypo/hypertension; decreased respiratory rate; tachycardia or bradycardia; cardiac dysrhythmias; seizures.	4012.2 Signs & Symptoms · p.141
	ALL	Listed differential: respiratory depression; insecticides (organophosphates); solvents, cleaning agents; cardiac medications; stimulants; depressants.	4012.2 Differential Diagnosis · p.141
☐	**ALL**	PEDIATRIC: most pediatric patients with respiratory depression do not have narcotic overdose; they are either septic or have respiratory failure.	4012.2 Pediatric Considerations · p.141

2 ASSESSMENT

☐	**ALL**	Consider other causes of altered mental status such as hypoglycemia, head trauma, sepsis, and stroke.	4012.2 Clinical Pearls · p.142

Required assessment elements (substance, amount, time of ingestion, vital signs, glucose, ECG): NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE: OPIOID OVERDOSE

☐	**EMR+**	If respirations are impaired or there is suspicion of narcotic overdose — ADULT: Naloxone up to 4 mg IN (half dose per nostril). *Condition / note: 8033 Medical Control: No.*	4012.2 Tx Algorithm, EMR block · p.141 8033 Adult Dosing · p.289
☐	**EMR+**	PEDIATRIC Naloxone: less than or equal to 20 kg, 0.1 mg/kg IN (max dose 2 mg), may repeat x one; greater than 20 kg, 2 mg IN, may repeat as needed. *Condition / note: 8033 does not say 'x one' (see U-4012-2).*	4012.2 Tx Algorithm, EMR block · p.141 8033 Pediatric Dosing · p.289
☐	**EMR+**	Consider patient restraint before administration of Naloxone. *Condition / note: Restraint method is not stated (see U-4012-3).*	4012.2 Tx Algorithm, EMR block · p.141

EMT LEVEL AND ABOVE

	EMT+	No additional orders at this level (source).	4012.2 Tx Algorithm, EMT block · p.141

AEMT LEVEL AND ABOVE: OPIOID OVERDOSE

☐	**AEMT+**	If the patient has a pulse, Naloxone should be administered before inserting an ETT.	4012.2 Tx Algorithm, AEMT block · p.141 8033 Precautions · p.289
☐	**AEMT+**	ADULT Naloxone: up to 4 mg IN, 2 mg IV or 4 mg IM. *Condition / note: 8033: IM 'if no IV' (see U-4012-2).*	4012.2 Tx Algorithm, AEMT block · p.141 8033 Adult Dosing · p.289
☐	**AEMT+**	PEDIATRIC Naloxone: less than or equal to 20 kg, 0.1 mg/kg IN, IV, IM (max dose 2 mg), may repeat x one; greater than 20 kg, 2 mg IN, IV, IM, may repeat as needed. Slow IV is preferred but IN or IM may be given before IV is established.	4012.2 Tx Algorithm, AEMT block · p.141 8033 Pediatric Dosing · p.289

AEMT LEVEL AND ABOVE: STIMULANT OVERDOSE WITH CHEST PAIN (ADULT)

☐	**AEMT+**	Stimulant overdose (cocaine, methamphetamines, amphetamines, crack cocaine) with chest pain: Nitroglycerin 0.4 mg SL, if SBP greater than 100, every 5 minutes to a total of three doses with vital signs between doses. *Condition / note: 8034 age and contraindication wording not in 4012 (see U-4012-4).*	4012.2 Tx Algorithm, AEMT block · p.141 8034 Adult Dosing / Indications · p.291
☐	**AEMT+**	Midazolam 10 mg IN (5 mg in each nostril) OR 2.5 mg slow IV OR 5 mg IM.	4012.2 Tx Algorithm, AEMT block · p.141 8031 Adult Dosing · p.285
☐	**AEMT+**	For patients greater than 69 y/o, reduce dosing for sedatives and analgesics to one half of the adult doses.	4012.2 Tx Algorithm, AEMT block · p.141 8031 Adult Dosing · p.285

PARAMEDIC

☐	**PM** **MCP ORDER**	Tricyclic antidepressant overdose (may be evidenced by bradycardia, tachycardia, hypotension and prolongation of the QRS complex; risk of rapid deterioration or sudden onset V Fib is high) — MCP ORDER: ADULT Sodium Bicarbonate 100 mEq slow IV; PEDIATRIC 1 mEq/kg slow IV.	4012.2 Tx Algorithm, Paramedic block; Consult · p.142 8042 Adult / Pediatric Dosing · p.307
☐	**PM** **MCP ORDER**	MCP ORDER: repeat Sodium Bicarbonate for persistent QRS prolongation — ADULT 50 mEq slow IV; PEDIATRIC 0.5 mEq/kg slow IV.	4012.2 Tx Algorithm, Paramedic block · p.142 8042 Adult / Pediatric Dosing · p.307
☐	**PM** **MCP ORDER**	Calcium channel blocker overdose — MCP ORDER: ADULT Calcium Chloride 1 Gm slow IV; PEDIATRIC Calcium Chloride 0.2 ml/kg (20 mg/kg) slow IV (max dose 500 mg).	4012.2 Tx Algorithm, Paramedic block; Consult · p.142 8006 Adult / Pediatric Dosing · p.235

Beta blocker overdose treatment (the Consult line names Beta Blocker antidotes; no treatment is printed): NOT SPECIFIED IN SOURCE — VERIFY.

Treatment for other poisonings / overdoses (e.g., depressants, solvents, organophosphates, cardiac medications): NOT SPECIFIED IN SOURCE — VERIFY (only naloxone, stimulant chest pain, tricyclic and calcium channel blocker items are printed).

PEARLS WITH TREATMENT RELEVANCE

☐	**ALL**	Naloxone is not felt to be effective in the reversal of cardiac arrest from opioid overdose. Airway control, ventilation, and quality CPR are still the mainstay of treatment in cardiac arrest management.	4012.2 Clinical Pearls · p.142
☐	**ALL**	Naloxone is not indicated in the management of newborns as it can cause withdrawal and secondary issues.	4012.2 Clinical Pearls · p.142 8033 Contraindications · p.289
☐	**ALL**	Ondansetron (Zofran) is NOT to be given prophylactically with Naloxone.	4012.2 Clinical Pearls · p.142
☐	**ALL**	Button battery ingestion (lithium) — not in EMS scope of practice: pure honey to coat the battery may be administered ONLY by the patient or the patient's caregivers if honey is at the scene and they wish to do so; about 10 ml every 10 minutes, up to 6 times; contraindicated in patients less than 12 months old; document as 'self or family administered', not administered by the provider. *Condition / note: See U-4012-8.*	4012.2 Clinical Pearls · p.142 4012.2 Pediatric Considerations · p.141

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 4012 ITSELF)

☐	**EMR+**	NALOXONE (8033): contraindications hypersensitivity; newborn patients. Use with caution in narcotic-dependent patients (withdrawal); may not reverse hypotension. Onset is two minutes; if no response two minutes after dosing, give additional doses. IN doses are half in each nostril. Transport by EMS is encouraged even if the patient becomes responsive.	8033 Contraindications / Precautions · p.289
☐	**AEMT+**	NITROGLYCERIN (8034): hypersensitivity; hypotension; sexual enhancement drugs (Viagra, Cialis, Levitra) in last 24 hours; taking Revatio; head injury; use only on patients greater than 25 years old or prescribed nitroglycerin; not indicated in pediatric patients.	8034 Contraindications / Precautions / Pediatric · p.291
☐	**AEMT+**	MIDAZOLAM (8031): contraindication respiratory distress; can cause respiratory depression; monitor respirations and ventilate if necessary; provide continuous cardiac monitoring, EtCO2 and pulse oximetry with sedated patients.	8031 Contraindications / Precautions · p.285
☐	**PM**	SODIUM BICARBONATE (8042): precautions metabolic alkalosis; hypoxia; hypernatremia; seizures; tissue sloughing. CALCIUM CHLORIDE (8006): do not administer with sodium bicarbonate (precipitate); flush tubing between drugs; bradycardia, hypotension, severe local necrosis after IV infiltration.	8042 Precautions · p.307 8006 Precautions · p.235

EMT DELEGATION — from the source level tables

MAY	EMT (and EMR): ADULT Naloxone up to 4 mg IN (half dose per nostril), may repeat in 2 minutes; PEDIATRIC Naloxone IN by weight (20 kg or less 0.1 mg/kg IN, max 2 mg; greater than 20 kg 2 mg IN); titrate to adequate respirations; consider patient restraint before naloxone (4012.2 EMR block, PDF p.141; 8033 Medical Control: No). EMT block: 'No additional orders at this level.'
MCP ORDER	EMT, MCP ORDER (diamond): EMS personnel should contact and confer with MCP for direction on suspected poisonings (4012.1.a; Consult).
MAY	EMT may assist the advanced provider with IV access, fluids, saline locks, EKG lead placement and drug bag access under direct supervision (1005.3, PDF p.21 — see Batch 1). Honey for button battery ingestion may be administered ONLY by the patient or caregivers, never by the provider (Pearls).

NOT LISTED AT EMT LEVEL	IV / IM naloxone; nitroglycerin and midazolam for stimulant chest pain; sodium bicarbonate for tricyclic overdose; calcium chloride for calcium channel blocker overdose.

4 REASSESSMENT / RESPONSE

☐	**EMR+**	Titrate Naloxone to adequate respirations. *Condition / note: No maximum cumulative dose is stated (see U-4012-2).*	4012.2 Tx Algorithm, EMR block · p.141
☐	**EMR+**	ADULT: may repeat Naloxone doses in 2 minutes. AEMT / Paramedic: consider repeat IV dosing if no or inadequate response is noted.	4012.2 Tx Algorithm, EMR / AEMT blocks · p.141
☐	**AEMT+**	When given IV or IN, the onset of action for Naloxone is approximately 2 minutes. PEDIATRIC, IN route: if respirations don't improve after 2 minutes, establish IV and administer an IV dose.	4012.2 Tx Algorithm, AEMT block · p.141 4012.2 Clinical Pearls · p.142
☐	**AEMT+**	Stimulant chest pain: repeat Midazolam 5 mg IN (2.5 mg in each nostril) OR 2.5 mg slow IV OR 5 mg IM for unrelieved chest pain. *Condition / note: 4012 gives no interval; 8031 does (see U-4012-5).*	4012.2 Tx Algorithm, AEMT block · p.141 8031 Adult Dosing · p.285

Reassessment interval, response-to-treatment criteria and monitoring after tricyclic / calcium channel blocker therapy: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**ALL MCP ORDER**	For guidance on suspected poisonings contact MCP. Calcium Channel Blocker, Beta Blocker and Tricyclic antidotes in this protocol are by MCP order only.	4012.2 Consult · p.142
☐	**ALL**	Provide the receiving facility the substance information (SDS, container, label or image).	4012.1.c · p.141

Transport, destination and notification requirements for overdose / poisoning, and handoff content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for overdose / poisoning (substance, amount, time, naloxone doses and response; honey as 'self or family administered' is the only documentation instruction printed): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4012-1	4012.1.a / 4012.2 Consult (p.141)	4012.1.a is a diamond item: 'EMS personnel should contact and confer with MCP for direction on suspected poisonings'
U-4012-2	4012.2 vs 8033 (p.289)	Naloxone: 4012 adult AEMT line 'up to 4 mg IN, 2mg IV or 4 mg IM'; 8033 says up to 4 mg IN or 2 mg IV, 'If no IV, up...
U-4012-3	4012.2 EMR block (p.141)	'Consider patient restraint before administration of Naloxone' — restraint method, authority and safety conditions...
U-4012-4	4012.2 vs 8034 (p.291)	Stimulant chest pain: 4012 gives nitroglycerin 'if SBP >100', 3 doses
U-4012-5	4012.2 AEMT block vs 8031 (p.285)	Stimulant chest pain midazolam: 4012 repeat line has no interval
U-4012-6	4012.2 Paramedic block vs 8042 / 8006 (p.307)	Tricyclic and calcium channel blocker treatments are diamond items (MCP order; 8042 and 8006 Medical Control: Yes)
U-4012-7	4012.2 Consult (p.142)	The Consult says 'Calcium Channel Blocker, Beta Blocker and Trycyclic antidotes in this protocol are by MCP order...
U-4012-8	4012.2 Pearls (p.142)	Button battery honey: 'not in EMS scope of practice' yet described with administration details (10 ml every 10...
U-4012-9	4012 scope (p.141)	Only opioid, stimulant chest pain, tricyclic and calcium channel blocker treatments are printed
U-4012-10	4012 edition (p.141)	'Last modified' Mar. 9, 2025 — earlier than the other protocols reviewed; not listed in Appendix A (G-1)

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4013 — RESPIRATORY DISTRESS / PULMONARY EDEMA

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Respiratory Distress / Pulmonary Edema	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4013 — Respiratory Distress / Pulmonary Edema	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.143 (+ monograph 8034 p.291, 8002 p.227, 8023 p.269)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (4013.1)

☐	**EMR+**	Signs & symptoms: cyanosis; clammy skin; presence/absence of fever; coughing; wheezing; labored breathing; diaphoresis; pitting edema; bilateral lower lobe rales; tachypnea; apprehension; jugular vein distension (JVD); inability to talk.	4013.1 Signs & Symptoms · p.143
	ALL	Listed differential: myocardial infarction; congestive heart failure; asthma; anaphylaxis; aspiration; chronic obstructive pulmonary disease; pleural effusion; pneumonia; pulmonary embolus; pericardial tamponade. Pediatric Considerations: None.	4013.1 Differential / Pediatric · p.143

2 ASSESSMENT

☐	**EMR+**	Evaluate breath sounds.	4013.1 Tx Algorithm, EMR block · p.143
☐	**EMR+**	Obtain pulse oximetry reading.	4013.1 Tx Algorithm, EMR block · p.143
☐	**EMR+**	Obtain capnography reading.	4013.1 Tx Algorithm, EMR block · p.143
☐	**EMT+** **{ } OPTION**	{Obtain and transmit 12 Lead EKG}.	4013.1 Tx Algorithm, EMT block · p.143
☐	**PM**	Cardiac monitoring. *Condition / note: Printed at Paramedic level only (see U-4013-3).*	4013.1 Tx Algorithm, Paramedic block · p.143
	ALL	Breath-sound guide (Pearls) — Clear: treat cause (e.g., MI, pulmonary embolism, metabolic disturbance, hyperventilation). Wheezes: treat cause (e.g., pulmonary edema, FBAO, asthma, allergic reaction). Rales: treat cause (e.g., pulmonary edema, pneumonia). Diminished or absent — unilateral: treat cause (e.g., pneumothorax, hemothorax, pneumonia, surgically removed lung); bilateral: treat cause (e.g., respiratory failure, COPD, asthma).	4013.1 Clinical Pearls · p.143
	ALL	Pearl: pneumonia may look like CHF with pulmonary edema; the pneumonia patient is often dehydrated and has an elevated temperature.	4013.1 Clinical Pearls · p.143

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	Provide high flow O2. *Condition / note: Differs from SpO2-titrated oxygen in 2008 / 4017 (see U-4013-1).*	4013.1 Tx Algorithm, EMR block · p.143

EMT LEVEL AND ABOVE

☐	**EMT+**	ADULT: if pulmonary edema, then Continuous Positive Pressure Airway (CPAP). *Condition / note: No settings or contraindications in 4013 (see U-4013-2).*	4013.1 Tx Algorithm, EMT block · p.143

AEMT LEVEL AND ABOVE

☐	**AEMT+**	ADULT, if pulmonary edema: CPAP use is encouraged prior to the initiation of drug therapy.	4013.1 Tx Algorithm, AEMT block · p.143
☐	**AEMT+**	ADULT, if patient has SBP greater than 100: Nitroglycerin 0.4 mg SL up to 3, 1 every 5 minutes. *Condition / note: 8034 age / contraindication wording not in 4013 (see U-4013-4). 8034 Medical Control for AEMT: No.*	4013.1 Tx Algorithm, AEMT block · p.143 8034 Adult Dosing / Indications · p.291

PARAMEDIC

☐	**PM** **{ } OPTION**	If pulmonary edema: CPAP or {Bi-PAP} use is encouraged prior to the initiation of drug therapy.	4013.1 Tx Algorithm, Paramedic block · p.143
☐	**PM**	If pulmonary edema: consider need for possible early endotracheal intubation. *Condition / note: 1008 not reproduced (G-3).*	4013.1 Tx Algorithm, Paramedic block · p.143
☐	**ALL**	Where indicated, consider Albuterol 2.5 mg and Ipratropium 0.5 mg nebulized simultaneously in line with CPAP. *Condition / note: Level and indications not stated (see U-4013-5).*	4013.1 Clinical Pearls · p.143 8002 Adult Dosing · p.227 8023 Adult Dosing · p.269

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 4013 ITSELF)

☐	**AEMT+**	NITROGLYCERIN (8034): hypersensitivity; hypotension; sexual enhancement drugs (Viagra, Cialis, Levitra) in last 24 hours; taking Revatio; head injury; use only on patients greater than 25 years old or prescribed nitroglycerin; not indicated in pediatric patients; indicated for pulmonary edema with systolic BP over 100 mmHg (AEMT and Paramedic).	8034 Indications / Contraindications / Precautions · p.291
☐	**EMT+**	ALBUTEROL (8002): prior hypersensitive reaction; cardiac dysrhythmias associated with tachycardia. IPRATROPIUM (8023): caution in narrow-angle glaucoma and lactating mothers.	8002 Contraindications · p.227 8023 Precautions · p.269

EMT DELEGATION — from the source level tables	
MAY	EMT: evaluate breath sounds; obtain pulse oximetry and capnography; provide high flow O2; call for transport; {obtain and transmit 12 Lead EKG}; ADULT with pulmonary edema, CPAP (4013.1 EMR / EMT blocks, PDF p.143). Consult: none.
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Nitroglycerin (AEMT block; 8034 Medical Control: EMT Yes for chest pain only); cardiac monitoring (Paramedic block); Bi-PAP; early intubation; nebulized albuterol / ipratropium in line with CPAP (in Pearls, level not printed).
NOTE	No pediatric treatment is printed (Pediatric Considerations: None); the CPAP and nitroglycerin lines carry adult markers.

4 REASSESSMENT / RESPONSE

Reassessment interval, response-to-treatment criteria, SpO2 / EtCO2 targets and a worsening pathway (e.g., failure of CPAP): NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport.	4013.1 Tx Algorithm, EMR block · p.143
☐	**EMT+ { } OPTION**	{Transmit the 12 Lead EKG} (EMT block).	4013.1 Tx Algorithm, EMT block · p.143
☐	**ALL**	Consult: none printed.	4013.1 Consult · p.143

Destination, receiving-facility notification and handoff content for respiratory distress / pulmonary edema: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for respiratory distress / pulmonary edema: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4013-1	4013.1 EMR block (p.143)	'Provide high flow O2' with no SpO2 target. Other protocols reviewed (2008, 4017) direct oxygen only below 94% and...
U-4013-2	4013.1 EMT / Paramedic (p.143)	CPAP: 'If Pulmonary Edema, then CPAP' (adult marker, EMT) — no indications beyond pulmonary edema, no...
U-4013-3	4013.1 AEMT vs Paramedic blocks (p.143)	Cardiac monitoring is printed in the Paramedic block only, while the AEMT block gives nitroglycerin
U-4013-4	4013.1 vs 8034 (p.291)	Nitroglycerin: 4013 'SBP greater than 100 ... up to 3, 1 every 5 minutes'; 8034 lists 'Hypotension' with no number,...
U-4013-5	4013.1 Pearls (p.143)	'Where indicated, consider administering Albuterol 2.5 mg and Ipratropium 0.5 mg, nebulized simultaneously inline...
U-4013-6	4013.1 scope (p.143)	No treatment is printed for non-pulmonary-edema causes (pneumonia, pulmonary embolus, pneumothorax,...

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4014 — SEIZURES

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Seizures	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4014 — Seizures	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.145-146 (+ 1005 p.22; monographs 8024 pp.271-272, 8029 p.281, 8031 p.285; Appendix A pp.322-323)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (4014.1)

☐	**EMR+**	Signs & symptoms: decreased mental status; sleepiness; incontinence; observed seizure activity; evidence of trauma.	4014.1 Signs & Symptoms · p.145
	ALL	Listed differential: head trauma; tumor; metabolic, hepatic or renal failure; hypoxia; electrolyte abnormality; drugs, medications; infection/fever; alcohol withdrawal; eclampsia; stroke/TIA; hyperthermia; psychogenic non-epileptic seizures. Pediatric Considerations: None.	4014.1 Differential / Pediatric · p.145

2 ASSESSMENT

☐	**EMR+ { } OPTION**	Obtain Pulse Oximeter and {Capnography} reading.	4014.1 Tx Algorithm, EMR block · p.145
☐	**EMR+**	Perform blood glucose check (1005 General Patient Management). *Condition / note: 4014 itself only gives the conditional 'if glucose less than 60' (see U-4014-4).*	1005.4 Tx Algorithm, EMR block · p.22
☐	**AEMT+**	Cardiac monitor.	4014.1 Tx Algorithm, AEMT block · p.145
☐	**ALL**	History: description of seizures, areas of body involved, and duration; other known medical history (e.g., head injury, diabetes, drugs, alcohol, stroke, heart disease, recent fever or illness, possible toxicological agents).	4014.1 Clinical Pearls · p.146

Required vital signs, post-ictal assessment elements and pregnancy-status question: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	BVM and nasopharyngeal airway during seizure as needed.	4014.1 Tx Algorithm, EMR block · p.145
☐	**EMR+**	Maintain normothermia. *Condition / note: Method not stated.*	4014.1 Tx Algorithm, EMR block · p.145

EMT LEVEL AND ABOVE

☐	**EMT+**	If glucose less than 60, or strong suspicion of hypoglycemia despite glucometer readings, then follow 4008 Hypoglycemia/Hyperglycemia. *Condition / note: 4008 not reproduced (G-3).*	4014.1 Tx Algorithm, EMT block · p.145
☐	**EMT+**	Place the patient in the recovery position during assessment and transport.	4014.1 Tx Algorithm, EMT block · p.145

AEMT LEVEL AND ABOVE: ACTIVELY SEIZING ADULT

☐	**AEMT+**	For patients greater than 69 y/o, reduce dosing for sedatives and analgesics to one half of the adult doses. *Condition / note: See U-4014-2.*	4014.1 Tx Algorithm, AEMT block · p.145 8031 Adult Dosing · p.285
☐	**AEMT+**	ADULT: Midazolam 10 mg IN (5 mg in each nostril), OR Midazolam 2.5 mg slow IV, OR Midazolam 5 mg IM.	4014.1 Tx Algorithm, AEMT block · p.145 8031 Adult Dosing · p.285

AEMT LEVEL AND ABOVE: ACTIVELY SEIZING PEDIATRIC

☐	**AEMT+**	PEDIATRIC: Midazolam 0.2 mg/kg IN (max IN dose 10 mg) OR Midazolam 0.1 mg/kg slow IV (max IV dose 2.5 mg) OR Midazolam 0.2 mg/kg IM (max IM dose 5 mg).	4014.1 Tx Algorithm, AEMT block · p.145 8031 Pediatric Dosing · p.285

PARAMEDIC

☐	**PM**	ADULT: if seizing pregnant patient with gestation greater than 20 weeks or postpartum less than 12 weeks, administer Magnesium Sulfate 4 grams (whole bag) infused with macro-drip tubing over 20 min.	4014.1 Tx Algorithm, Paramedic block · p.146 8029 Adult Dosing / Indications · p.281

☐	**PM**	PEDIATRIC: if the seizing pregnant or postpartum patient is less than 16 years old but meets the criteria above, administer Magnesium Sulfate 4 grams (whole bag) infused with macro-drip tubing over 20 min.	4014.1 Tx Algorithm, Paramedic block · p.146 8029 Pediatric Dosing · p.281

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 4014 ITSELF)

☐	**AEMT+**	MIDAZOLAM (8031): contraindication respiratory distress; can cause respiratory depression; monitor respirations and ventilate if necessary; provide continuous cardiac monitoring, EtCO2 and pulse oximetry with sedated patients; geriatric and debilitated patients require lower doses.	8031 Contraindications / Precautions · p.285
☐	**AEMT+**	KETAMINE (8024) contraindications: suspected cardiac chest pain; hypertensive crisis; when significant elevations in BP might prove harmful (acute myocardial infarction, angina pectoris, aortic dissection). Do not reduce ketamine dosing for geriatric patients in RSE.	8024 Contraindications · p.272 8024 Adult Dosing · p.271
☐	**PM**	MAGNESIUM SULFATE (8029): hypersensitivity; any heart block except first degree; caution if SBP less than 90 mmHg; continuous cardiac monitoring and frequent vital signs required; can cause respiratory depression; hypotension, bradycardia and conduction disturbances may occur if given rapidly.	8029 Contraindications / Precautions · p.281

EMT DELEGATION — from the source level tables

MAY	EMT (and EMR): BVM and nasopharyngeal airway during seizure as needed; maintain normothermia; pulse oximeter and {capnography}; if glucose less than 60 or strong suspicion of hypoglycemia follow 4008; place the patient in the recovery position during assessment and transport (4014.1 EMR / EMT blocks, PDF p.145). Consult: none.
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Cardiac monitor; midazolam by any route; ketamine for RSE (AEMT block); magnesium sulfate for seizing pregnant / postpartum patients (Paramedic block).
NOTE	EMT role in 4014 is airway, glucose, positioning and monitoring; there is no EMT medication and no MCP order printed.

4 REASSESSMENT / RESPONSE

IF STILL SEIZING, REPEAT MIDAZOLAM (ADULT)

☐	**AEMT+**	Repeat Midazolam 5 mg IN (2.5 mg in each nostril) after 10 minutes. *Condition / note: 8031 prints '5 g IN' (see U-4014-3).*	4014.1 Tx Algorithm, AEMT block · p.145 8031 Adult Dosing · p.285
☐	**AEMT+**	OR repeat Midazolam 2.5 mg slow IV after 5 minutes.	4014.1 Tx Algorithm, AEMT block · p.145
☐	**AEMT+**	OR repeat Midazolam 5 mg IM after 10 minutes.	4014.1 Tx Algorithm, AEMT block · p.145

IF STILL SEIZING, REPEAT MIDAZOLAM (PEDIATRIC)

☐	**AEMT+**	Repeat Midazolam 0.2 mg/kg IN (max IN dose 5 mg) after 10 minutes.	4014.1 Tx Algorithm, AEMT block · p.145 8031 Pediatric Dosing · p.285
☐	**AEMT+**	OR repeat Midazolam 0.1 mg/kg slow IV (max IV dose 2.5 mg) after 5 minutes.	4014.1 Tx Algorithm, AEMT block · p.145
☐	**AEMT+**	OR repeat Midazolam 0.2 mg/kg IM (max IM dose 5 mg) after 10 minutes.	4014.1 Tx Algorithm, AEMT block · p.145

REFRACTORY STATUS EPILEPTICUS (RSE) UNTREATED BY TWO DOSES OF MIDAZOLAM

☐	**AEMT+**	ADULT: repeat any of the Midazolam doses / routes above, OR consider Ketamine 100 mg IV/IO or (if no IV/IO) then 250 mg IM. *Condition / note: See U-4014-1.*	4014.1 Tx Algorithm, AEMT block · p.145 8024 Indications / Adult Dosing · p.271 8031 Adult Dosing · p.285
☐	**AEMT+**	PEDIATRIC: repeat any of the pediatric Midazolam doses / routes above. *Condition / note: No pediatric ketamine option is printed (see U-4014-1).*	4014.1 Tx Algorithm, AEMT block · p.145 8031 Pediatric Dosing · p.285
	ALL	Pearl: RSE is defined as seizures that continue despite treatment with first-line medications. Post-medication signs and symptoms could include: generalized tonic/clonic seizures lasting longer than 5 minutes; focal motor seizures lasting longer than 10 minutes; absences lasting longer than 10 minutes; recurrent seizures without a lucid interval.	4014.1 Clinical Pearls · p.146

Post-ictal reassessment interval, criteria for 'still seizing', and response-to-treatment criteria: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMT+**	Place patient in the recovery position during assessment and transport.	4014.1 Tx Algorithm, EMT block · p.145
☐	**ALL**	Consult: none printed.	4014.1 Consult · p.146

Transport decision, destination, notification and handoff content for seizures (including post-seizure refusal): NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for seizures (seizure duration, times of each midazolam dose, pregnancy gestation / postpartum weeks): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4014-1	4014.1 RSE vs Pearls vs 8024 / 8031 (p.146)	Two RSE definitions: the algorithm uses 'untreated by two doses of Midazolam'; the Pearls define RSE as 'seizures...
U-4014-2	4014.1 AEMT block vs 8031 / 8024 (p.285)	Geriatric reduction: 4014 prints the half-dose rule above the adult midazolam line with no exceptions
U-4014-3	8031 Adult Dosing (p.285)	8031 prints the adult IN repeat as 'Repeat 5 g IN (after 10 min.)'; 4014 says 'Repeat Midazolam 5 mg IN (2.5 mg in...
U-4014-4	4014.1 EMT block (p.145)	Glucose: only 'If glucose less than 60, or there is strong suspicion of hypoglycemia' is given; no instruction to...
U-4014-5	4014.1 Paramedic block vs 8029 (p.281)	Magnesium: adult criteria 'gestation greater than 20 weeks or postpartum less than 12 weeks'; the pediatric line...
U-4014-6	4014.1 Tx Algorithm (p.145)	'Actively seizing' has no minimum duration; no instruction for a patient who has stopped seizing (other than...
U-4014-7	4014 Appendix A (p.322)	Appendix A (pp.322-323) lists new 4014 RSE dosing and RSE signs and symptoms (reflected here)

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

REQUIRES HUMAN VERIFICATION

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

Global issues (all protocols in this file)

ID	Topic	Issue	Clarification needed
G-1	Edition / effective date	Cover (PDF p.1): 'Last Update 07/01/2026'. Appendix A (PDF p.321): '2026-2028 Protocol Changes', Effective Nov. 1, 2026. Each protocol prints 'Effective: June 1, 2021' and its own 'Last modified'. 1001.1.h: after release of a new version, all previous versions are invalid. Today is Oct. 30, 2026.	Confirm with agency clinical leadership which edition is in force today and on Nov. 1, 2026 before any field use.
G-2	Level mapping method	Provider levels come from the colored side bars of each Clinical Management Table (grey EMR, orange EMT, green AEMT, blue Paramedic). Text extraction cannot carry that information, so levels were read from rendered page images and cross-checked against Appendix A and the monographs.	Reviewer should spot-check level tags against the printed color bars.
G-3	Cross-referenced protocols not reproduced	Referenced but NOT built or reviewed in Batches 1-3: 1005 (only quoted lines used), 1007 (CPAP/Bi-PAP), 1008/1009 (airway), 1012 (IO), 1014 (only quoted lines), 3001, 3015, 3016, 3017, 3018, 3020, 4005, 4008, 4011, 4016 (shock), 8035 (norepinephrine), 8036 (fluids), 7012/7013, 3019/3021. Batch 2 built 2010, 2011, 3002, 4002, 4017; Batch 3 built 2001, 4003, 4012, 4013, 4014.	Tell me which to add in later batches.
G-4	Agency options and Medical Director approval	Items in { } are agency/Medical Director options (1001.5.d.v) and 1001.4.d says nothing may be used without Medical Director pre-approval. This checklist cannot know which options your department has adopted. 1001.4.e-f: protocol is used as clinically indicated; not every standing order applies to every patient.	Agency Medical Director to mark adopted options.
G-5	Monograph level columns	Drug monographs (8000 series) print level tabs as column headers; the text extraction does not show which column each row sits in. Medication level statements in the checklist rely on the protocol tables, with monographs used for doses, contraindications and Medical Control lines.	Reviewer to confirm any level-specific monograph statement.
G-6	Status	This checklist has NOT been reviewed or approved by agency clinical leadership and does not authorize care. Reviewer sign-off lines in Section 7 are intentionally blank.	Complete the review sign-off.
G-7	Monograph cross-check scope and numbering	Batch 2 doses were compared with monographs 8001, 8002, 8003, 8005, 8006/8007, 8013, 8017, 8018, 8019, 8023, 8024, 8026, 8029, 8030, 8031 and 8042 (Medical Control lines, dosing, contraindications). Footer / header numbers are inconsistent in the source: the Calcium Gluconate page header reads 8007 but its footer reads '8006 - Calcium Gluconate'; the Norepinephrine page header reads 8035 but its footer reads '8034 - Norepinephrine'; Nitroglycerin is 8034. Monograph level columns were not read (G-5).	Reviewer: confirm drug and number mapping; no clinical content was inferred from the numbering.

2001 Resuscitation Guidelines and Field Termination

ID	Source location	Issue	Clarification needed
U-2001-1	2001.2 Pediatric vs criteria (p.45)	'FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS', yet the criteria say '18 years or older'. 1005.4 defines pediatric as less than 16 years, so ages 16-17 are not clearly addressed. 2001.1.d says the standards apply to adults and pediatrics.	Medical Director: rule for ages 16-17.
U-2001-2	2001.2 EMT vs AEMT blocks (p.45)	Two field-termination provisions: (1) EMT block, diamond: 'If no ALS equipment is available at the scene, and transport time ... will exceed 20 minutes, field termination may be considered'; (2) AEMT block, diamond: six criteria incl. advanced airway and vascular access. The EMT provision does not say whether the six criteria also apply, who contacts MCP, or what the EMT reports. The Consult script is marked 'A' (adult) for AEMT or Paramedic only.	Clarify the EMT pathway.
U-2001-3	2001.2 AEMT block (p.45)	'Prolonged resuscitation' if PEA 'greater than 40 per minute'; field termination only for asystole or PEA 'less than 40' — PEA at exactly 40 is not covered. The EtCO2 criterion reads 'upward trending or persistent EtCO2 greater than or equal to 20 mmHg, refractory to VF or VT': whether it applies only to VF / VT or to all rhythms is unclear, and no EtCO2 value or timing is a termination criterion.	Medical Director clarification.
U-2001-4	2001.2 AEMT block (p.45)	'Following all appropriate efforts' has no minimum duration, cycle count or intervention list in 2001; those are in 2002, 2003 and 2005 (see U-2005 items in Batch 1, including the 2005 Consult reference to field termination). 'No signs of neurological function' lists three examples only.	Confirm minimum effort standard.
U-2001-5	2001.2 Tx Algorithm (p.45)	ROSC and STEMI-with-witnessed-arrest go to an interventional facility if transport time is less than 30 minutes; Pearls say consider aeromedical transport for transports greater than 30 minutes with ROSC. The list of interventional facilities (7012 / 7013, see U-2008-8) is not reproduced. Exactly 30 minutes is not addressed.	Confirm facility list and the 30-minute boundary.
U-2001-6	2001.2 AEMT block (p.45)	'Profound hypothermia' is undefined and the destination is a Trauma Center; 1003.3.a excludes hypothermic patients from non-initiation. 3016 Hypothermia is not reproduced.	Add 3016 in a later batch.
U-2001-7	2001.1.c / 2001.2 (p.45)	'Prioritize on-scene resuscitation ... unless clinically indicated' — no list of clinical indications to transport earlier. EMT block says 'If no higher-level provider is available, then transport', which could conflict with 'best chance is at the scene' for EMT-only crews. Appendix A (PDF p.321) lists 2001.1.c and 2001.1.d as new language.	Medical Director clarification.
U-2001-8	2001 edition (p.45)	'Last modified' is May 26, 2026 (the other Batch 2 protocols show May 16, 2026). See G-1 for the edition question.	Confirm edition in force.

4003 Asthma / Emphysema / COPD

ID	Source location	Issue	Clarification needed
U-4003-1	4003.1 vs 4002.2 (p.122)	4003 prints no MCP-order requirement for the EMT's nebulized Albuterol / Ipratropium, consistent with Appendix A (p.322) and monographs 8002 / 8023 (Medical Control: No). 4002 Consult still says 'EMT needs MCP orders to administer breathing treatments' (U-4002-1). The same drugs, same EMT, two protocols disagree.	Medical Director: confirm; update 4002 text.

ID	Source location	Issue	Clarification needed
U-4003-2	4003.1 EMT / Paramedic (p.121)	CPAP is listed for 'any patient who is bronchial constricted' (EMT) and Bi-PAP is a bracketed { } option (Paramedic). No indications, contraindications, pressure settings or patient-selection criteria are in 4003. 1007 Basic Airway Maintenance (which describes CPAP and {BiPAP} as non-invasive ventilation) is not reproduced.	Add 1007 in a later batch; agency: is Bi-PAP adopted?
U-4003-3	4003.1 AEMT block vs 8017 / 8018 (p.259)	Epinephrine: 'Asthmatics in severe distress (NOT for emphysema or COPD)' — 'severe distress' is not defined; the maximum number of repeat doses is not stated; no MCP order is printed (8017 / 8018 Medical Control: initial dose No; follow-up dosing for AEMT and Paramedic No). Pediatric note says 30 kg or more gets 'Epinephrine 0.5 mg IM' while the algorithm also lists EpiPens; 8017 pediatric says 'greater than 30 kg' (see U-4002-3).	Medical Director: define severe distress and dose limit.
U-4003-4	4003.1 AEMT block (p.121)	Needle decompression: 'consider ... in the presence of auto-PEEP or hyperinflation' — how these are recognized is not stated. It is printed in the green (AEMT) block (level read from the color bar, G-2). No catheter size is given in 4003 (3002 gives 14-gauge or larger). Bilateral decompression in cardiac arrest vs 'only the affected sides' when unstable with diminished sounds.	Reviewer: confirm level; Medical Director: indications and equipment.
U-4003-5	4003.1 AEMT block (p.121)	Post-intubation ventilation rate (adult 8-10 / pediatric 10-15 per minute) is conditional on being able to 'adequately oxygenate'; no oxygenation target is given. 1008 Advanced Airway Management is not reproduced.	Medical Director clarification.
U-4003-6	4003.1 vs 8029 (p.281)	Magnesium: 4003 says 'refractory to Albuterol, Ipratropium, and Epinephrine' (adult marker only, diamond, MCP order). 8029 Indications say 'refractory to Duo-Neb, and Epinephrine and Solu-Medrol' and its Medical Control says Adult: Yes when given for asthma refractory to primary medications. Whether Solu-Medrol must be given first is not stated in 4003. No pediatric asthma magnesium dose in 4003 or 8029 (pediatric listing is seizing pregnant / postpartum only).	Confirm 'refractory' definition.
U-4003-7	4003.1 Paramedic block (p.121)	Solu-Medrol is listed without timing relative to other treatments or an indication restriction (8030: asthma, COPD, emphysema, allergic reaction; 'Do not initiate an IV only to administer this medication'). 4003 gives no instruction for emphysema / COPD epinephrine other than 'NOT for emphysema or COPD'.	Confirm sequence.
U-4003-8	4003 Appendix A (p.322)	Appendix A (pp.322-323) lists new 4003 text for the EMT (no MCP for nebulized treatments; in-line with CPAP). Both are reflected here. Monograph level columns were not read (G-5). 'Last modified' May 16, 2026 (G-1).	Confirm edition in force.

4012 Overdose / Poisoning

ID	Source location	Issue	Clarification needed
U-4012-1	4012.1.a / 4012.2 Consult (p.141)	4012.1.a is a diamond item: 'EMS personnel should contact and confer with MCP for direction on suspected poisonings'. 8033 Medical Control for naloxone is 'No'. It is not stated whether MCP contact must happen before naloxone, nitroglycerin or midazolam, or only for the poisonings that have no printed treatment. The Consult says 'For guidance on suspected poisonings contact MCP'.	Medical Director: timing and scope of the MCP contact.
U-4012-2	4012.2 vs 8033 (p.289)	Naloxone: 4012 adult AEMT line 'up to 4 mg IN, 2mg IV or 4 mg IM'; 8033 says up to 4 mg IN or 2 mg IV, 'If no IV, up to 4 mg IM'. Pediatric 20 kg or less: 4012 'may repeat x one'; 8033 says 'Titrate dosing to adequate respirations, repeat as needed'. No maximum cumulative dose or endpoint other than 'adequate respirations'. 4012 and 8033 define pediatric dosing by weight, 1005.4 defines pediatric as less than 16 years.	Confirm repeat limits and IM wording.
U-4012-3	4012.2 EMR block (p.141)	'Consider patient restraint before administration of Naloxone' — restraint method, authority and safety conditions are not stated in 4012 (4005 Behavioral Emergencies - Combative Patients was not reviewed).	Medical Director clarification; review 4005.
U-4012-4	4012.2 vs 8034 (p.291)	Stimulant chest pain: 4012 gives nitroglycerin 'if SBP >100', 3 doses. 8034 lists hypotension (no number), PDE-5 drugs within 24 hours, Revatio, head injury, and 'use only on patients greater than 25 years old or prescribed nitroglycerin' (see U-2008-1). 8034 Medical Control for AEMT / Paramedic: No. Order of nitroglycerin and midazolam is not stated.	Confirm that 8034 cautions apply.
U-4012-5	4012.2 AEMT block vs 8031 (p.285)	Stimulant chest pain midazolam: 4012 repeat line has no interval. 8031 Adult Dosing covers 'seizures, chemical restraint, or chest pain in stimulant overdose' with repeats after 10 min (IN), 5 min (slow IV), 10 min (IM) — but prints the IN repeat as '5 g IN' (a typographical error for mg). 8031 indication for stimulant chest pain is 'adults only'; 4012 has no pediatric option.	Confirm interval and correct 8031 typographical error.
U-4012-6	4012.2 Paramedic block vs 8042 / 8006 (p.307)	Tricyclic and calcium channel blocker treatments are diamond items (MCP order; 8042 and 8006 Medical Control: Yes). 4012 says 'slow IV'; 8042 and 8006 say 'IV'. 8007 Calcium Gluconate exists only for a calcium chloride shortage; 4012 names only calcium chloride. 'Tricyclic' / 'Calcium Channel Blocker' example lists are in the Pearls (spelling as printed, e.g., 'Trycyclic' in the Consult).	Confirm administration time.
U-4012-7	4012.2 Consult (p.142)	The Consult says 'Calcium Channel Blocker, Beta Blocker and Trycyclic antidotes in this protocol are by MCP order only', but 4012 contains no beta blocker antidote. Glucagon or other agents are not printed in this protocol.	Medical Director: is a beta blocker treatment intended?
U-4012-8	4012.2 Pearls (p.142)	Button battery honey: 'not in EMS scope of practice' yet described with administration details (10 ml every 10 minutes up to 6 times; document as 'self or family administered'). Age limit 12 months appears in the Pediatric Considerations ('greater to or equal to 12 months') and in the Pearls ('contraindicated in patients less than 12 months old'). No instructions for transport, imaging or airway are printed.	Confirm intent.
U-4012-9	4012 scope (p.141)	Only opioid, stimulant chest pain, tricyclic and calcium channel blocker treatments are printed. Organophosphate / nerve agent (8005, 6005), cyanide (3008), carbon monoxide (3006) and other toxidromes are in other protocols not reproduced. The Pediatric Considerations say most pediatric respiratory depression is not narcotic.	Do not treat this as a complete poisoning checklist.
U-4012-10	4012 edition (p.141)	'Last modified' Mar. 9, 2025 — earlier than the other protocols reviewed; not listed in Appendix A (G-1). Monograph level columns not read (G-5).	Confirm edition in force.

4013 Respiratory Distress / Pulmonary Edema

ID	Source location	Issue	Clarification needed
U-4013-1	4013.1 EMR block (p.143)	'Provide high flow O2' with no SpO2 target. Other protocols reviewed (2008, 4017) direct oxygen only below 94% and titrated to 94%. 4013 also directs pulse oximetry and capnography but gives no values.	Medical Director: confirm intended oxygen strategy.
U-4013-2	4013.1 EMT / Paramedic (p.143)	CPAP: 'If Pulmonary Edema, then CPAP' (adult marker, EMT) — no indications beyond pulmonary edema, no contraindications, settings or pediatric guidance in 4013; 1007 not reproduced. Bi-PAP is a bracketed { } option (agency option unknown).	Add 1007 in a later batch; agency: Bi-PAP adopted?
U-4013-3	4013.1 AEMT vs Paramedic blocks (p.143)	Cardiac monitoring is printed in the Paramedic block only, while the AEMT block gives nitroglycerin. Levels were read from the color bars (G-2).	Reviewer: confirm level reading.
U-4013-4	4013.1 vs 8034 (p.291)	Nitroglycerin: 4013 'SBP greater than 100 ... up to 3, 1 every 5 minutes'; 8034 lists 'Hypotension' with no number, PDE-5 / Revatio exclusions, head injury, and 'greater than 25 years old or prescribed nitroglycerin' (see U-2008-1, U-2008-3); 8034 indication is 'Pulmonary edema with systolic BP over 100 mmHg' — 'over' vs 'greater than' is the same, but exactly 100 is not addressed.	Confirm age limit and contraindications.
U-4013-5	4013.1 Pearls (p.143)	'Where indicated, consider administering Albuterol 2.5 mg and Ipratropium 0.5 mg, nebulized simultaneously inline with CPAP' appears in Clinical Pearls with no level and no indication (wheezing? bronchospasm?). Appendix A (p.322) lists it as new Pearl language. 4003 prints the same combination at the EMT level with no MCP order.	Confirm level and indication.
U-4013-6	4013.1 scope (p.143)	No treatment is printed for non-pulmonary-edema causes (pneumonia, pulmonary embolus, pneumothorax, hyperventilation); the Pearls say 'treat cause'. Pediatric Considerations: None. 4003, 4002, 2008 and 1008 hold related treatments.	Do not treat this as a complete respiratory checklist.

4014 Seizures

ID	Source location	Issue	Clarification needed
U-4014-1	4014.1 RSE vs Pearls vs 8024 / 8031 (p.146)	Two RSE definitions: the algorithm uses 'untreated by two doses of Midazolam'; the Pearls define RSE as 'seizures that continue despite treatment with first-line medications' with duration thresholds (generalized tonic/clonic longer than 5 minutes, focal motor and absences longer than 10 minutes, recurrent without a lucid interval). 'Repeat any of the Midazolam doses/routes' has no maximum number or cumulative limit (8031: 'repeat at second dose levels/routes'; Appendix A p.323: a third dose was added). Ketamine is adult only; 8024 pediatric dosing has no seizure indication.	Medical Director: unify the RSE definition and limits.
U-4014-2	4014.1 AEMT block vs 8031 / 8024 (p.285)	Geriatric reduction: 4014 prints the half-dose rule above the adult midazolam line with no exceptions. 8031 exceptions are '{RSI or sedate-to-intubate}, combative patients, pacing or cardioversion' (seizures are not an exception); 8024 says do not reduce for RSE seizures. 4014 does not say how to halve IN doses (e.g., the 10 mg IN dose).	Confirm geriatric dosing for seizures.
U-4014-3	8031 Adult Dosing (p.285)	8031 prints the adult IN repeat as 'Repeat 5 g IN (after 10 min.)'; 4014 says 'Repeat Midazolam 5 mg IN (2.5 mg in each nostril) after 10 minutes'. Treated as a typographical error in 8031; not corrected silently.	Correct 8031.
U-4014-4	4014.1 EMT block (p.145)	Glucose: only 'If glucose less than 60, or there is strong suspicion of hypoglycemia' is given; no instruction to check, no units. 1005.4 EMR block ('Perform blood glucose check') is cited for the check. 4008 is not reproduced (also referenced in 4017).	Add 4008 in a later batch.
U-4014-5	4014.1 Paramedic block vs 8029 (p.281)	Magnesium: adult criteria 'gestation greater than 20 weeks or postpartum less than 12 weeks'; the pediatric line applies to a pregnant / postpartum patient 'less than 16 years old but meets criteria above'. 8029 Medical Control lists Adult: Yes only for asthma, so none for seizures. 8029 contraindications (heart block) are not in 4014. Appendix A (p.323) lists the 8029 change from 6 to 12 weeks postpartum. 4011 Obstetrical Emergencies not reproduced.	Confirm applicability of monograph cautions.
U-4014-6	4014.1 Tx Algorithm (p.145)	'Actively seizing' has no minimum duration; no instruction for a patient who has stopped seizing (other than positioning, glucose and monitoring); no treatment for febrile or eclamptic seizures other than the magnesium line. {Capnography} is a bracketed { } option. 'Maintain normothermia' has no method.	Medical Director clarification.
U-4014-7	4014 Appendix A (p.322)	Appendix A (pp.322-323) lists new 4014 RSE dosing and RSE signs and symptoms (reflected here). 'Last modified' May 16, 2026 (G-1); monograph level columns not read (G-5).	Confirm edition in force.

AUDIT SUMMARY

Automated source audit at build: 137 checklist items in this file - every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures; whole-document audit 472 items, 0 failures); 157 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 21 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

SOURCE-VERIFICATION TABLES

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

One row per checklist line. 'Traced' = the cited section and PDF page were located and the item's key phrases and numbers were machine-matched to that page; levels were read from the rendered color bars. Human review is still pending. Level column: lowest level at which the source lists the item (cumulative).

2001 Resuscitation Guidelines and Field Termination — PDF p.45 (+ 1003 p.17, 1005 p.21; Appendix A p.321)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2001-1.01	A patient's BEST CHANCE for resuscitation is at the scene with...	2001.1.a (p.45)	ALL	-	Traced; review pending
2001-1.02	Paramedics are expected to provide ALS resuscitative care at...	2001.1.b (p.45)	PM	-	Traced; review pending
2001-1.03	All providers should prioritize on-scene resuscitation efforts...	2001.1.c (p.45)	ALL	'Unless clinically indicated' is not defined (see U-2001-7).	Traced; review pending
2001-1.04	These standards apply to all patient populations, including...	2001.1.d (p.45)	ALL	-	Traced; review pending
2001-1.05	Signs: pulseless and apneic; does NOT meet the Non-initiation...	2001.2 Signs & Symptoms (p.45)	EMR+	Non-initiation criteria are in 1003 (Batch 1).	Traced; review pending
2001-1.06	Listed differential: meets Non-initiation of Care Guideline.	2001.2 Differential Diagnosis (p.45)	ALL	-	Traced; review pending
2001-1.07	PEDIATRIC: FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC...	2001.2 Pediatric Considerations (p.45)	ALL	Age boundary vs the 18-year criterion: see U-2001-1.	Traced; review pending
2001-2.01	Rhythm check, CPR quality targets, airway / IV / IO steps and...	-		-	NOT SPECIFIED — VERIFY
2001-3.01	The EMR will continue resuscitation until the patient is handed...	2001.2 Tx Algorithm, EMR block (p.45)	EMR+	-	Traced; review pending
2001-3.02	The EMT will continue resuscitation until the patient is handed...	2001.2 Tx Algorithm, EMT block (p.45)	EMT+	-	Traced; review pending
2001-3.03	Patients will require PROLONGED resuscitation efforts if: they...	2001.2 Tx Algorithm, AEMT block (p.45)	AEMT +	See U-2001-3.	Traced; review pending
2001-3.04	Pearl: there are situations where resuscitation may take 30...	2001.2 Clinical Pearls (p.45)	ALL	-	Traced; review pending
2001-3.05	Pearl: in pseudo PEA, the patient may not be in true cardiac...	2001.2 Clinical Pearls (p.45)	ALL	-	Traced; review pending
2001-4.01	MCP ORDER: following all appropriate efforts, field termination...	2001.2 Tx Algorithm, AEMT block (p.45)	AEMT +	'All appropriate efforts' is not defined; PEA exactly 40 is not covered (see U-2001-3, U-2001-4).	Traced; review pending
2001-4.02	MCP ORDER (EMT block): if NO ALS equipment is available at the...	2001.2 Tx Algorithm, EMT block (p.45)	EMT+	Relationship to the AEMT-block criteria is not stated (see U-2001-2).	Traced; review pending
2001-4.03	Reassessment interval, number of resuscitation cycles or...	-		-	NOT SPECIFIED — VERIFY
2001-5.01	If no higher-level provider is available, then transport.	2001.2 Tx Algorithm, EMT block (p.45)	EMT+	-	Traced; review pending
2001-5.02	Patient with ROSC should be transported to an interventional...	2001.2 Tx Algorithm, EMT block (p.45)	EMT+	-	Traced; review pending
2001-5.03	If arrest is due to profound hypothermia, rapidly transport to...	2001.2 Tx Algorithm, AEMT block (p.45)	AEMT +	'Profound' is not defined (see U-2001-6).	Traced; review pending
2001-5.04	Transport to a cardiac interventional facility if transport...	2001.2 Tx Algorithm, Paramedic block (p.45)	PM	-	Traced; review pending
2001-5.05	Pearl: consider aeromedical transport for transports greater...	2001.2 Clinical Pearls (p.45)	ALL	-	Traced; review pending
2001-5.06	The duration of the resuscitation.	2001.2 Consult (p.45)	AEMT +	-	Traced; review pending
2001-5.07	How long the patient may have been in arrest prior to EMS...	2001.2 Consult (p.45)	AEMT +	-	Traced; review pending
2001-5.08	Whether it was a witnessed or unwitnessed event.	2001.2 Consult (p.45)	AEMT +	-	Traced; review pending
2001-5.09	The current EtCO2.	2001.2 Consult (p.45)	AEMT +	EtCO2 is reported but is not a stated termination criterion.	Traced; review pending
2001-5.10	The presenting rhythm.	2001.2 Consult (p.45)	AEMT +	-	Traced; review pending
2001-5.11	Receiving-facility notification and handoff content for arrest...	-		-	NOT SPECIFIED — VERIFY
2001-6.01	Documentation / closeout elements for resuscitation and field...	-		-	NOT SPECIFIED — VERIFY

4003 Asthma / Emphysema / COPD — PDF pp.121-122 (+ monographs 8002 p.227, 8017 p.257, 8018 p.259, 8023 p.269, 8026 p.275, 8029 p.281, 8030 p.283; Appendix A pp.322-323)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4003-1.01	Signs & symptoms: shortness of breath; pursed lip breathing...	4003.1 Signs & Symptoms (p.121)	EMR+	-	Traced; review pending
4003-1.02	Listed differential: anaphylaxis; aspiration; pleural effusion...	4003.1 Differential Diagnosis (p.121)	ALL	-	Traced; review pending
4003-1.03	PEDIATRIC: younger patients may exhibit nasal flaring...	4003.1 Pediatric Considerations (p.121)	ALL	-	Traced; review pending
4003-1.04	PEDIATRIC: the protocol lists patients under 15 kg as...	4003.1 Pediatric Considerations (p.121)	ALL	See U-4003-3.	Traced; review pending
4003-2.01	Required assessment elements (history, medication use, vital...	-		-	NOT SPECIFIED — VERIFY
4003-3.01	Provide O2 as needed.	4003.1 Tx Algorithm, EMR block (p.121)	EMR+	-	Traced; review pending
4003-3.02	If the patient develops wheezing, assist with taking their...	4003.1 Tx Algorithm, EMT block (p.121); 8002 Adult Dosing (p.227); 8023 Adult Dosing (p.269)	EMT+	No MCP order is printed (Appendix A: requirement removed). 4002 still prints one (see U-4003-1).	Traced; review pending
4003-3.03	For any patient who is bronchial constricted: CPAP. If...	4003.1 Tx Algorithm, EMT block (p.121)	EMT+	CPAP settings / contraindications are not in 4003 (see U-4003-2).	Traced; review pending
4003-3.04	If the patient is intubated, Albuterol 2.5 mg by nebulizer into...	4003.1 Tx Algorithm, AEMT block (p.121)	AEMT +	-	Traced; review pending
4003-3.05	After intubation of an asthma patient, limit the ventilation...	4003.1 Tx Algorithm, AEMT block (p.121)	AEMT +	-	Traced; review pending
4003-3.06	Consider needle decompression in the presence of auto-PEEP or...	4003.1 Tx Algorithm, AEMT block (p.121)	AEMT +	Level read from the green color bar (see U-4003-4).	Traced; review pending
4003-3.07	Decompression sites: fourth or fifth intercostal space in the...	4003.1 Tx Algorithm, AEMT block (p.121)	AEMT +	-	Traced; review pending
4003-3.08	Asthmatics in severe distress (NOT for emphysema or COPD): if...	4003.1 Tx Algorithm, AEMT block (p.121); 8018 Indications / Adult Dosing (p.259); 8017 Indications (p.257)	AEMT +	EMT may not treat asthma with epinephrine (8017 / 8018). 'Severe distress' undefined (see U-4003-3).	Traced; review pending
4003-3.09	PEDIATRIC: equal to or greater than 15 kg and less than 30 kg...	4003.1 Tx Algorithm, AEMT block (p.121); 8018 Pediatric Dosing (p.259)	AEMT +	-	Traced; review pending
4003-3.10	If a conscious patient requires intubation — ADULT: Lidocaine...	4003.1 Tx Algorithm, Paramedic block (p.121); 8026 Adult / Pediatric Dosing (p.275)	PM	-	Traced; review pending
4003-3.11	For any persistent bronchial constriction consider CPAP or...	4003.1 Tx Algorithm, Paramedic block (p.121)	PM	-	Traced; review pending
4003-3.12	Solu-Medrol: ADULT 125 mg IV. PEDIATRIC 2 mg/kg IV, max dose...	4003.1 Tx Algorithm, Paramedic block (p.121); 8030 Adult / Pediatric Dosing (p.283)	PM	-	Traced; review pending
4003-3.13	MCP ORDER: bronchoconstriction due to asthma refractory to...	4003.1 Tx Algorithm, Paramedic block (p.121); 4003.1 Consult (p.122); 8029 Adult Dosing / Medical Control (p.281)	PM	8029 states refractory to 'Duo-Neb, Epinephrine and Solu-Medrol' (see U-4003-6).	Traced; review pending
4003-3.14	Pearl: no significant change in patient condition in the field...	4003.1 Clinical Pearls (p.122)	ALL	-	Traced; review pending
4003-3.15	ALBUTEROL (8002): prior hypersensitive reaction to albuterol...	8002 Contraindications / Precautions (p.227); 8023 Contraindications / Precautions (p.269)	EMT+	-	Traced; review pending
4003-3.16	EPINEPHRINE (8017, 8018): contraindications none in the...	8018 Contraindications / Precautions (p.259); 8017 Contraindications (p.257)	AEMT +	-	Traced; review pending
4003-3.17	MAGNESIUM SULFATE (8029): hypersensitivity; any heart block...	8029 Contraindications / Precautions (p.281)	PM	-	Traced; review pending
4003-3.18	SOLU-MEDROL (8030): contraindications none in emergency...	8030 Contraindications / Precautions (p.283)	PM	-	Traced; review pending
4003-3.19	LIDOCAINE (8026): hypersensitivity; second or third degree...	8026 Contraindications / Precautions (p.275)	PM	-	Traced; review pending
4003-4.01	EMT: may repeat Albuterol 2.5 mg nebulized two times.	4003.1 Tx Algorithm, EMT block (p.121); 8002 Adult Dosing (p.227)	EMT+	-	Traced; review pending
4003-4.02	Asthmatics in severe distress: may repeat Epinephrine (1:1,000)...	4003.1 Tx Algorithm, AEMT block (p.121)	AEMT +	Maximum number of doses not stated (see U-4003-3).	Traced; review pending
4003-4.03	Reassessment interval, response-to-treatment criteria and...	-		-	NOT SPECIFIED — VERIFY
4003-5.01	Call for transport.	4003.1 Tx Algorithm, EMR block (p.121)	EMR+	-	Traced; review pending
4003-5.02	Transport unless ALS intercept is less than 5 minutes.	4003.1 Tx Algorithm, EMT block (p.121)	EMT+	-	Traced; review pending
4003-5.03	A patient who has received a breathing treatment should be...	4003.1 Clinical Pearls (p.122)	ALL	-	Traced; review pending
4003-5.04	Paramedic needs MCP orders to administer Magnesium Sulfate to...	4003.1 Consult (p.122)	PM	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4003-5.05	Destination, notification and handoff content for respiratory...	-		-	NOT SPECIFIED — VERIFY
4003-6.01	Documentation / closeout elements for asthma / COPD (e.g...	-		-	NOT SPECIFIED — VERIFY

4012 Overdose / Poisoning — PDF pp.141-142 (+ monographs 8006 p.235, 8031 p.285, 8033 p.289, 8034 p.291, 8042 p.307)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4012-1.01	MCP ORDER (diamond): EMS personnel should contact and confer...	4012.1.a (p.141); 4012.2 Consult (p.142)	ALL	Timing relative to naloxone is not stated (see U-4012-1).	Traced; review pending
4012-1.02	Poison Control is intended for use by the general public.	4012.1.b (p.141)	ALL	-	Traced; review pending
4012-1.03	If possible, provide the receiving facility all available...	4012.1.c (p.141)	ALL	-	Traced; review pending
4012-1.04	Signs & symptoms: mental status changes; hypo/hypertension...	4012.2 Signs & Symptoms (p.141)	EMR+	-	Traced; review pending
4012-1.05	Listed differential: respiratory depression; insecticides...	4012.2 Differential Diagnosis (p.141)	ALL	-	Traced; review pending
4012-1.06	PEDIATRIC: most pediatric patients with respiratory depression...	4012.2 Pediatric Considerations (p.141)	ALL	-	Traced; review pending
4012-2.01	Consider other causes of altered mental status such as...	4012.2 Clinical Pearls (p.142)	ALL	-	Traced; review pending
4012-2.02	Required assessment elements (substance, amount, time of...	-		-	NOT SPECIFIED — VERIFY
4012-3.01	If respirations are impaired or there is suspicion of narcotic...	4012.2 Tx Algorithm, EMR block (p.141); 8033 Adult Dosing (p.289)	EMR+	8033 Medical Control: No.	Traced; review pending
4012-3.02	PEDIATRIC Naloxone: less than or equal to 20 kg, 0.1 mg/kg IN...	4012.2 Tx Algorithm, EMR block (p.141); 8033 Pediatric Dosing (p.289)	EMR+	8033 does not say 'x one' (see U-4012-2).	Traced; review pending
4012-3.03	Consider patient restraint before administration of Naloxone.	4012.2 Tx Algorithm, EMR block (p.141)	EMR+	Restraint method is not stated (see U-4012-3).	Traced; review pending
4012-3.04	No additional orders at this level (source).	4012.2 Tx Algorithm, EMT block (p.141)	EMT+	-	Traced; review pending
4012-3.05	If the patient has a pulse, Naloxone should be administered...	4012.2 Tx Algorithm, AEMT block (p.141); 8033 Precautions (p.289)	AEMT +	-	Traced; review pending
4012-3.06	ADULT Naloxone: up to 4 mg IN, 2 mg IV or 4 mg IM.	4012.2 Tx Algorithm, AEMT block (p.141); 8033 Adult Dosing (p.289)	AEMT +	8033: IM 'if no IV' (see U-4012-2).	Traced; review pending
4012-3.07	PEDIATRIC Naloxone: less than or equal to 20 kg, 0.1 mg/kg IN...	4012.2 Tx Algorithm, AEMT block (p.141); 8033 Pediatric Dosing (p.289)	AEMT +	-	Traced; review pending
4012-3.08	Stimulant overdose (cocaine, methamphetamines, amphetamines...	4012.2 Tx Algorithm, AEMT block (p.141); 8034 Adult Dosing / Indications (p.291)	AEMT +	8034 age and contraindication wording not in 4012 (see U-4012-4).	Traced; review pending
4012-3.09	Midazolam 10 mg IN (5 mg in each nostril) OR 2.5 mg slow IV OR...	4012.2 Tx Algorithm, AEMT block (p.141); 8031 Adult Dosing (p.285)	AEMT +	-	Traced; review pending
4012-3.10	For patients greater than 69 y/o, reduce dosing for sedatives...	4012.2 Tx Algorithm, AEMT block (p.141); 8031 Adult Dosing (p.285)	AEMT +	-	Traced; review pending
4012-3.11	Tricyclic antidepressant overdose (may be evidenced by...	4012.2 Tx Algorithm, Paramedic block; Consult (p.142); 8042 Adult / Pediatric Dosing (p.307)	PM	-	Traced; review pending
4012-3.12	MCP ORDER: repeat Sodium Bicarbonate for persistent QRS...	4012.2 Tx Algorithm, Paramedic block (p.142); 8042 Adult / Pediatric Dosing (p.307)	PM	-	Traced; review pending
4012-3.13	Calcium channel blocker overdose — MCP ORDER: ADULT Calcium...	4012.2 Tx Algorithm, Paramedic block; Consult (p.142); 8006 Adult / Pediatric Dosing (p.235)	PM	-	Traced; review pending
4012-3.14	Beta blocker overdose treatment (the Consult line names Beta...	-		-	NOT SPECIFIED — VERIFY
4012-3.15	Treatment for other poisonings / overdoses (e.g., depressants...	-		-	NOT SPECIFIED — VERIFY
4012-3.16	Naloxone is not felt to be effective in the reversal of cardiac...	4012.2 Clinical Pearls (p.142)	ALL	-	Traced; review pending
4012-3.17	Naloxone is not indicated in the management of newborns as it...	4012.2 Clinical Pearls (p.142); 8033 Contraindications (p.289)	ALL	-	Traced; review pending
4012-3.18	Ondansetron (Zofran) is NOT to be given prophylactically with...	4012.2 Clinical Pearls (p.142)	ALL	-	Traced; review pending
4012-3.19	Button battery ingestion (lithium) — not in EMS scope of...	4012.2 Clinical Pearls (p.142); 4012.2 Pediatric Considerations (p.141)	ALL	See U-4012-8.	Traced; review pending
4012-3.20	NALOXONE (8033): contraindications hypersensitivity; newborn...	8033 Contraindications / Precautions (p.289)	EMR+	-	Traced; review pending
4012-3.21	NITROGLYCERIN (8034): hypersensitivity; hypotension; sexual...	8034 Contraindications / Precautions / Pediatric (p.291)	AEMT +	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4012-3.22	MIDAZOLAM (8031): contraindication respiratory distress; can...	8031 Contraindications / Precautions (p.285)	AEMT +	-	Traced; review pending
4012-3.23	SODIUM BICARBONATE (8042): precautions metabolic alkalosis...	8042 Precautions (p.307); 8006 Precautions (p.235)	PM	-	Traced; review pending
4012-4.01	Titrate Naloxone to adequate respirations.	4012.2 Tx Algorithm, EMR block (p.141)	EMR+	No maximum cumulative dose is stated (see U-4012-2).	Traced; review pending
4012-4.02	ADULT: may repeat Naloxone doses in 2 minutes. AEMT /...	4012.2 Tx Algorithm, EMR / AEMT blocks (p.141)	EMR+	-	Traced; review pending
4012-4.03	When given IV or IN, the onset of action for Naloxone is...	4012.2 Tx Algorithm, AEMT block (p.141); 4012.2 Clinical Pearls (p.142)	AEMT +	-	Traced; review pending
4012-4.04	Stimulant chest pain: repeat Midazolam 5 mg IN (2.5 mg in each...	4012.2 Tx Algorithm, AEMT block (p.141); 8031 Adult Dosing (p.285)	AEMT +	4012 gives no interval; 8031 does (see U-4012-5).	Traced; review pending
4012-4.05	Reassessment interval, response-to-treatment criteria and...	-		-	NOT SPECIFIED — VERIFY
4012-5.01	For guidance on suspected poisonings contact MCP. Calcium...	4012.2 Consult (p.142)	ALL	-	Traced; review pending
4012-5.02	Provide the receiving facility the substance information (SDS...	4012.1.c (p.141)	ALL	-	Traced; review pending
4012-5.03	Transport, destination and notification requirements for...	-		-	NOT SPECIFIED — VERIFY
4012-6.01	Documentation / closeout elements for overdose / poisoning...	-		-	NOT SPECIFIED — VERIFY

4013 Respiratory Distress / Pulmonary Edema — PDF p.143 (+ monograph 8034 p.291, 8002 p.227, 8023 p.269)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4013-1.01	Signs & symptoms: cyanosis; clammy skin; presence/absence of...	4013.1 Signs & Symptoms (p.143)	EMR+	-	Traced; review pending
4013-1.02	Listed differential: myocardial infarction; congestive heart...	4013.1 Differential / Pediatric (p.143)	ALL	-	Traced; review pending
4013-2.01	Evaluate breath sounds.	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
4013-2.02	Obtain pulse oximetry reading.	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
4013-2.03	Obtain capnography reading.	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
4013-2.04	{Obtain and transmit 12 Lead EKG}.	4013.1 Tx Algorithm, EMT block (p.143)	EMT+	-	Traced; review pending
4013-2.05	Cardiac monitoring.	4013.1 Tx Algorithm, Paramedic block (p.143)	PM	Printed at Paramedic level only (see U-4013-3).	Traced; review pending
4013-2.06	Breath-sound guide (Pearls) — Clear: treat cause (e.g., MI...	4013.1 Clinical Pearls (p.143)	ALL	-	Traced; review pending
4013-2.07	Pearl: pneumonia may look like CHF with pulmonary edema; the...	4013.1 Clinical Pearls (p.143)	ALL	-	Traced; review pending
4013-3.01	Provide high flow O2.	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	Differs from SpO2-titrated oxygen in 2008 / 4017 (see U-4013-1).	Traced; review pending
4013-3.02	ADULT: if pulmonary edema, then Continuous Positive Pressure...	4013.1 Tx Algorithm, EMT block (p.143)	EMT+	No settings or contraindications in 4013 (see U-4013-2).	Traced; review pending
4013-3.03	ADULT, if pulmonary edema: CPAP use is encouraged prior to the...	4013.1 Tx Algorithm, AEMT block (p.143)	AEMT +	-	Traced; review pending
4013-3.04	ADULT, if patient has SBP greater than 100: Nitroglycerin 0.4...	4013.1 Tx Algorithm, AEMT block (p.143); 8034 Adult Dosing / Indications (p.291)	AEMT +	8034 age / contraindication wording not in 4013 (see U-4013-4). 8034 Medical Control for AEMT: No.	Traced; review pending
4013-3.05	If pulmonary edema: CPAP or {Bi-PAP} use is encouraged prior to...	4013.1 Tx Algorithm, Paramedic block (p.143)	PM	-	Traced; review pending
4013-3.06	If pulmonary edema: consider need for possible early...	4013.1 Tx Algorithm, Paramedic block (p.143)	PM	1008 not reproduced (G-3).	Traced; review pending
4013-3.07	Where indicated, consider Albuterol 2.5 mg and Ipratropium 0.5...	4013.1 Clinical Pearls (p.143); 8002 Adult Dosing (p.227); 8023 Adult Dosing (p.269)	ALL	Level and indications not stated (see U-4013-5).	Traced; review pending
4013-3.08	NITROGLYCERIN (8034): hypersensitivity; hypotension; sexual...	8034 Indications / Contraindications / Precautions (p.291)	AEMT +	-	Traced; review pending
4013-3.09	ALBUTEROL (8002): prior hypersensitive reaction; cardiac...	8002 Contraindications (p.227); 8023 Precautions (p.269)	EMT+	-	Traced; review pending
4013-4.01	Reassessment interval, response-to-treatment criteria, SpO2 /...	-		-	NOT SPECIFIED — VERIFY
4013-5.01	Call for transport.	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
4013-5.02	{Transmit the 12 Lead EKG} (EMT block).	4013.1 Tx Algorithm, EMT block (p.143)	EMT+	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4013-5.03	Consult: none printed.	4013.1 Consult (p.143)	ALL	-	Traced; review pending
4013-5.04	Destination, receiving-facility notification and handoff...	-		-	NOT SPECIFIED — VERIFY
4013-6.01	Documentation / closeout elements for respiratory distress /...	-		-	NOT SPECIFIED — VERIFY

4014 Seizures — PDF pp.145-146 (+ 1005 p.22; monographs 8024 pp.271-272, 8029 p.281, 8031 p.285; Appendix A pp.322-323)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4014-1.01	Signs & symptoms: decreased mental status; sleepiness...	4014.1 Signs & Symptoms (p.145)	EMR+	-	Traced; review pending
4014-1.02	Listed differential: head trauma; tumor; metabolic, hepatic or...	4014.1 Differential / Pediatric (p.145)	ALL	-	Traced; review pending
4014-2.01	Obtain Pulse Oximeter and {Capnography} reading.	4014.1 Tx Algorithm, EMR block (p.145)	EMR+	-	Traced; review pending
4014-2.02	Perform blood glucose check (1005 General Patient Management).	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	4014 itself only gives the conditional 'if glucose less than 60' (see U-4014-4).	Traced; review pending
4014-2.03	Cardiac monitor.	4014.1 Tx Algorithm, AEMT block (p.145)	AEMT +	-	Traced; review pending
4014-2.04	History: description of seizures, areas of body involved, and...	4014.1 Clinical Pearls (p.146)	ALL	-	Traced; review pending
4014-2.05	Required vital signs, post-ictal assessment elements and...	-		-	NOT SPECIFIED — VERIFY
4014-3.01	BVM and nasopharyngeal airway during seizure as needed.	4014.1 Tx Algorithm, EMR block (p.145)	EMR+	-	Traced; review pending
4014-3.02	Maintain normothermia.	4014.1 Tx Algorithm, EMR block (p.145)	EMR+	Method not stated.	Traced; review pending
4014-3.03	If glucose less than 60, or strong suspicion of hypoglycemia...	4014.1 Tx Algorithm, EMT block (p.145)	EMT+	4008 not reproduced (G-3).	Traced; review pending
4014-3.04	Place the patient in the recovery position during assessment...	4014.1 Tx Algorithm, EMT block (p.145)	EMT+	-	Traced; review pending
4014-3.05	For patients greater than 69 y/o, reduce dosing for sedatives...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Adult Dosing (p.285)	AEMT +	See U-4014-2.	Traced; review pending
4014-3.06	ADULT: Midazolam 10 mg IN (5 mg in each nostril), OR Midazolam...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Adult Dosing (p.285)	AEMT +	-	Traced; review pending
4014-3.07	PEDIATRIC: Midazolam 0.2 mg/kg IN (max IN dose 10 mg) OR...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Pediatric Dosing (p.285)	AEMT +	-	Traced; review pending
4014-3.08	ADULT: if seizing pregnant patient with gestation greater than...	4014.1 Tx Algorithm, Paramedic block (p.146); 8029 Adult Dosing / Indications (p.281)	PM	-	Traced; review pending
4014-3.09	PEDIATRIC: if the seizing pregnant or postpartum patient is...	4014.1 Tx Algorithm, Paramedic block (p.146); 8029 Pediatric Dosing (p.281)	PM	-	Traced; review pending
4014-3.10	MIDAZOLAM (8031): contraindication respiratory distress; can...	8031 Contraindications / Precautions (p.285)	AEMT +	-	Traced; review pending
4014-3.11	KETAMINE (8024) contraindications: suspected cardiac chest...	8024 Contraindications (p.272); 8024 Adult Dosing (p.271)	AEMT +	-	Traced; review pending
4014-3.12	MAGNESIUM SULFATE (8029): hypersensitivity; any heart block...	8029 Contraindications / Precautions (p.281)	PM	-	Traced; review pending
4014-4.01	Repeat Midazolam 5 mg IN (2.5 mg in each nostril) after 10...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Adult Dosing (p.285)	AEMT +	8031 prints '5 g IN' (see U-4014-3).	Traced; review pending
4014-4.02	OR repeat Midazolam 2.5 mg slow IV after 5 minutes.	4014.1 Tx Algorithm, AEMT block (p.145)	AEMT +	-	Traced; review pending
4014-4.03	OR repeat Midazolam 5 mg IM after 10 minutes.	4014.1 Tx Algorithm, AEMT block (p.145)	AEMT +	-	Traced; review pending
4014-4.04	Repeat Midazolam 0.2 mg/kg IN (max IN dose 5 mg) after 10...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Pediatric Dosing (p.285)	AEMT +	-	Traced; review pending
4014-4.05	OR repeat Midazolam 0.1 mg/kg slow IV (max IV dose 2.5 mg)...	4014.1 Tx Algorithm, AEMT block (p.145)	AEMT +	-	Traced; review pending
4014-4.06	OR repeat Midazolam 0.2 mg/kg IM (max IM dose 5 mg) after 10...	4014.1 Tx Algorithm, AEMT block (p.145)	AEMT +	-	Traced; review pending
4014-4.07	ADULT: repeat any of the Midazolam doses / routes above, OR...	4014.1 Tx Algorithm, AEMT block (p.145); 8024 Indications / Adult Dosing (p.271); 8031 Adult Dosing (p.285)	AEMT +	See U-4014-1.	Traced; review pending
4014-4.08	PEDIATRIC: repeat any of the pediatric Midazolam doses / routes...	4014.1 Tx Algorithm, AEMT block (p.145); 8031 Pediatric Dosing (p.285)	AEMT +	No pediatric ketamine option is printed (see U-4014-1).	Traced; review pending
4014-4.09	Pearl: RSE is defined as seizures that continue despite...	4014.1 Clinical Pearls (p.146)	ALL	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4014-4.10	Post-ictal reassessment interval, criteria for 'still seizing'...	-		-	NOT SPECIFIED — VERIFY
4014-5.01	Place patient in the recovery position during assessment and...	4014.1 Tx Algorithm, EMT block (p.145)	EMT+	-	Traced; review pending
4014-5.02	Consult: none printed.	4014.1 Consult (p.146)	ALL	-	Traced; review pending
4014-5.03	Transport decision, destination, notification and handoff...	-		-	NOT SPECIFIED — VERIFY
4014-6.01	Documentation / closeout elements for seizures (seizure...	-		-	NOT SPECIFIED — VERIFY